MIZRAHI, Gaby (id #843, dob: 12/04/1928)

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From: P3 Referral Department - (702) 786-6620

P3 Referral Department processes at Risk/Urgent Referrals within 5-7 business days & Routine Referrals within 10-14 business days.
If additional labs, radiology films, or other testing is needed to process your referral, a member of our team will contact you.

03/19/2026

To Provider	From Provider
AVERY HOME HEALTH **5660 W FLAMINGO RD STE A** **LAS VEGAS, NV 89103** **Phone:** **Phone: (702) 416-0611** **Fax:** **Fax: (702) 899-4479**	**TERESA HONG, MD** **P3NV - 2831 Business Park** **2831 Business Park Ct Suite 130** **LAS VEGAS, NV 89128-9000** **Phone: (702) 844-4848** **Fax: (844) 898-1275**

Referral Order Information

Diagnosis	• **Dementia** **ICD-10: F03.90: Unspecified dementia, unspecified severity, without behavioral disturbance, psychotic disturbance, mood disturbance, and anxiety; E46: Unspecified protein-calorie malnutrition; E88.09: Other disorders of plasma-protein metabolism, not elsewhere classified**
Order Name	**Orders included: 1** **Dementia** **ICD-10: F03.90: Unspecified dementia, unspecified severity, without behavioral disturbance, psychotic disturbance, mood disturbance, and anxiety; E46: Unspecified protein-calorie malnutrition; E88.09: Other disorders of plasma-protein metabolism, not elsewhere classified** • **HOSPICE & PALLIATIVE MEDICINE REFERRAL** **Schedule Within: provider's discretion** **Note to Provider:** **has dementia; protein malnutrition; chronic afib/flutter; hypercoagulable state, CAD; CKD3b; hyperPTH due to CKD; parotid nodule; old MI; gyne prolapse; hx of falls;** **Referral processed**
Notes	**has dementia; protein malnutrition; chronic afib/flutter; hypercoagulable state, CAD; CKD3b; hyperPTH due to CKD; parotid nodule; old MI; gyne prolapse; hx of falls; Referral processed**

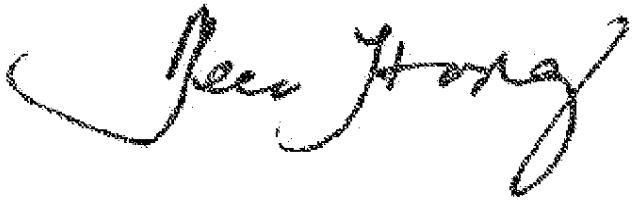
Patient Information

Patient Name	**MIZRAHI, GABY**
Sex - DOB - Age	**F 12/04/1928 97yo**
Address	**4355 S. DURANGO DR #105** **LAS VEGAS, NV 89147**
Phone	**H: (818) 239-6530** **M: (818) 239-6530**

MIZRAHI, Gaby (id #843, dob: 12/04/1928)

Primary Insurance	**Medicare-NV (Medicare)** **ID: 2MD6TC0XG75** **Policy Holder: MIZRAHI, GABY**
Secondary Insurance	**American Continental Insurance (Medicare Supplement)** **ID: 08AC135095** **Policy Holder: MIZRAHI, GABY**

Electronically Signed by: TERESA HONG, MD



TERESA HONG, MD

Encounters and Procedures

Clinical Encounter Summaries

Encounter Date: 01/14/2026 (Last amended by TERESA HONG, MD on 01/14/2026 at 2:41pm)

Patient

Name	MIZRAHI, GABY (97yo, F) ID# 843	**Appt. Date/Time**	01/14/2026 01:00PM
DOB	12/04/1928	**Service Dept.**	P3NV - 2831 Business Park
Provider	TERESA HONG, MD		
Insurance	Med Primary: MEDICARE-NV (MEDICARE) Insurance # : 2MD6TC0XG75 Med Secondary: AMERICAN CONTINENTAL INSURANCE (MEDICARE SUPPLEMENT) Insurance # : 08AC135095 Prescription: EXPRESS SCRIPTS - Member is eligible. details		

Chief Complaint

97 y/o female is present today for ER follow up due to a fall.
BAlvarez CCMA

Patient's Care Team

Other: SAMANTHA A SCHOENHAUS DO: 8285 W ARBY AVE STE 280, LAS VEGAS, NV 89113, Ph (702) 862-8862, Fax (702) 862-8774 NPI: 1295820330
Nephrologist: TEENA TANDON MD: 2420 PROFESSIONAL CT STE 150, LAS VEGAS, NV 89128, Ph (702) 853-0090, Fax (702) 853-0096 NPI: 1336349158
Cardiologist: HEART CENTER OF NEVADA (SUMMERLIN): 653 TOWN CENTER DR STE 410, LAS VEGAS, NV 89144, Ph (702) 804-0022, Fax (702) 331-9719
Podiatrist: FOOT AND ANKLE SPECIALISTS OF NEVADA: 7135 W SAHARA STE 201, LAS VEGAS, NV 89117, Ph (702) 878-2455, Fax (702) 851-7286
Urologist: SARAH RYAN MD: 3150 N TENAYA WAY STE 165, LAS VEGAS, NV 89128, Ph (702) 877-0814, Fax (702) 877-3238 NPI: 1659328391
Ophthalmologist: CENTER FOR SIGHT: 330 S RAMPART BLVD SUITE 360, LAS VEGAS, NV 89145, Ph (702)724-2020, Fax (702)724-2800 NPI: 1881946218
Nephrologist: KIDNEY SPECIALISTS OF SOUTHERN NEVADA: 653 TOWN CENTER DR BLG 2 STE 70, LAS VEGAS, NV 89144, Ph (702) 877-1887, Fax (702) 877-7141
Nutritionist: AM NUTRITION SERVICES: 1180 N TOWN CENTER DR STE 100, LAS VEGAS, NV 89144, Ph (623) 399-6825, Fax (702) 623-2921 NPI: 1003011602
Urologist: LAS VEGAS UROLOGY: 7500 SMOKE RANCH ROAD, LAS VEGAS, NV 89128, Ph (702) 233-0727, Fax (702) 233-4799 NPI: 1629054994
Dermatologist: VIVIDA DERMATOLOGY: 6460 MEDICAL CENTER ST SUITE 200 & 350, LAS VEGAS, NV 89148, Ph (702) 255-6647, Fax (702) 933-1444
Neurologist: GOBINDER CHOPRA MD: 6410 MEDICAL CENTER ST STE A, LAS VEGAS, NV 89148, Ph (702) 796-8500, Fax (702) 796-8502 NPI: 1013964626

MIZRAHI, Gaby (id #843, dob: 12/04/1928)

Patient's Pharmacies

COSTCO PHARMACY # 685 (ERX): 801 SOUTH PAVILION DR, LAS VEGAS, NV 89144, Ph (702) 352-2052, Fax (702) 352-2071
BLUE SKY DRUGS: PO BOX 29077, BELLINGHAM, WA 98228, Ph (866) 995-7387

Vitals

Ht:	5 ft Standing 01/14/2026 01:03 pm	**Wt:**	96.6 lbs With clothes 01/14/2026 01:13 pm	**BMI:**	18.9 01/14/2026 01:13 pm
BP:	110/70 sitting L arm 01/14/2026 01:13 pm	**O2Sat:**	97% Room Air at Rest 01/14/2026 01:13 pm	**Pain Scale:**	0 01/14/2026 01:13 pm
Pulse:	86 bpm regular 01/14/2026 01:13 pm	**RR:**	16 01/14/2026 01:05 pm	**T:**	97.3 F° temporal artery 01/14/2026 01:13 pm

Allergies

Reviewed Allergies
INFLUENZA VIRUS VACCINE, SPECIFIC
NKDA
Patient indicated NO allergies 09/18/2023

Medications

Reviewed Medications

allopurinol 100 mg tablet Take 0.5 tablet(s) every day by oral route.	01/14/26	entered
calcitriol 0.25 mcg capsule TAKE 1 CAPSULE (0.25 MCG) BY ORAL ROUTE MON, WED, FRI, SUN	12/13/24	filled
donepezil 5 mg tablet Take 1 tablet(s) every day by oral route.	12/02/25	prescribed
Eliquis 2.5 mg tablet TAKE ONE TABLET PO TWICE DAILY FOR A FIBRILLATION	01/14/26	prescribed
Ensure High Protein oral liquid 4-8 ml between meals tid.	10/20/23	prescribed
escitalopram 5 mg tablet TAKE ONE TABLET BY MOUTH ONCE DAILY	07/31/25	renewed
estradiol 0.01% (0.1 mg/gram) vaginal cream Insert every other day by vaginal route.	06/25/24	entered
K-Y Lubricating topical jelly apply perirectally bid	10/20/23	prescribed
Nystop 100,000 unit/gram topical powder	01/28/25	filled
pantoprazole 40 mg tablet,delayed release TAKE ONE TABLET BY MOUTH ONCE DAILY	06/03/24	filled
pravastatin 40 mg tablet TAKE 1 TABLET PO ONCE NIGHTLY FOR CHOLESTEROL	09/15/25	prescribed

SSD 1 % topical cream 10/20/23 filled
APPLY A 1/16 INCH (1.5 MM) THICK LAYER TO ENTIRE BURN AREA BY TOPICALROUTE 2 TIMES PER DAY

Stool Softener 100 mg capsule 05/21/24 renewed
TAKE ONE CAPSULE BY MOUTH ONCE DAILY

traZODone 50 mg tablet 07/20/25 renewed
1 TABLET 1HR PRIOR TO BEDTIME THEREAFTER

Vitamin D3 25 mcg (1,000 unit) capsule 06/26/18 entered
Take 1 capsule(s) every day by oral route.

Vaccines

Reviewed Vaccines
Declined

Problems

Reviewed Problems
- Atrial flutter, unspecified type - Onset: 01/12/2026
- Closed fracture of nasal bone with routine healing, subsequent encounter - Onset: 01/12/2026
- Closed displaced fracture of sixth cervical vertebra with routine healing, unspecified fracture morphology, subsequent encounter - Onset: 01/12/2026
- Chronic atrial fibrillation - Onset: 01/10/2025
- Parotid nodule - Onset: 01/12/2026
- Atrial fibrillation and flutter - Onset: 01/14/2026
- Underweight - Onset: 01/09/2025
- Old myocardial infarction - Onset: 03/24/2024
- Paroxysmal atrial fibrillation - Onset: 06/23/2024
- Chronic kidney disease stage 3B - Onset: 03/26/2024
- Screening for osteoporosis - Onset: 10/20/2023 - Dexa Dec 23, 2019 with osteopenia
- Candiduria - Onset: 10/20/2023
- Osteopenia - Onset: 10/20/2023 - Dexa Dec 23, 2019 with osteopenia
- Vaccination declined by caregiver - Onset: 10/20/2023
- Malnutrition (calorie) - Onset: 10/20/2023
- Bilateral pleural effusion - Onset: 07/13/2023 - CT Abdo and pelvis without contrast July 10, 2023 with: 1) BL renal cysts; 2 mm stone at mid pole L kidney 2) gallstone in the neck of the GB 1.7 cm 3) scattered diverticula without evidence of diverticulitis 4) trace bilateral pleural effusions 5) atherosclerosis of aorta
- Weight loss - Onset: 10/20/2023
- Diverticulosis of colon - Onset: 07/13/2023 - CT Abdo and pelvis without contrast July 10, 2023 with: 1) BL renal cysts; 2 mm stone at mid pole L kidney 2) gallstone in the neck of the GB 1.7 cm 3) scattered diverticula without evidence of diverticulitis 4) trace bilateral pleural effusions 5) atherosclerosis of aorta
- Kidney stone - Onset: 07/13/2023 - CT Abdo and pelvis without contrast July 10, 2023 with: 1) BL renal cysts; 2 mm stone at mid pole L kidney 2) gallstone in the neck of the GB 1.7 cm 3) scattered diverticula without evidence of diverticulitis 4) trace bilateral pleural effusions 5) atherosclerosis of aorta
- Anticoagulant therapy - Onset: 06/30/2023
- Tortuosity of branch of aortic arch - Onset: 06/30/2023 - CXR June 20, 2023 with cardiomegaly and mild BL infiltrates. moderate aortic tortuosity and calcification.
- Coronary atherosclerosis - Onset: 06/30/2023 - LHc June 20, 2023 with non-obstuctive CAD. likely transient occlusion of RCA CTA June 21, 2023 without PE; trace BL pleural effusions; thoracic aortic atherosclerotic plaque and coronary artery calcification Echo June 20, 2023 with EF 60-70%;
- Gallstone - Onset: 05/11/2023 - CT Abdo and pelvis without contrast July 10, 2023 with: 1) BL renal cysts; 2 mm stone at mid pole L kidney 2) gallstone in the neck of the GB 1.7 cm 3) scattered diverticula without evidence of diverticulitis 4) trace bilateral pleural effusions 5) atherosclerosis of aorta CT Abdo and pelvis Nov 23, 2022 with: 1) atherosclerosis of aorta root 2) GB with calcified gallstone at the neck with GB distension 3) BL renal cysts 4) urinary bladder moderately distended without filling defect or wall thickening
- Hyperuricemia - Onset: 05/11/2023
- History of gout - Onset: 05/11/2023
- Vaginal pessary in situ - Onset: 12/01/2022
- Osteoarthritis of joint of bilateral hands - Onset: 11/30/2020
- Nocturia - Onset: 11/30/2020
- Pain of left shoulder joint - Onset: 08/08/2022
- Spinal stenosis of lumbar region - Onset: 12/01/2022
- Hyperparathyroidism due to renal insufficiency - Onset: 06/26/2018
- Microscopic hematuria - Onset: 05/11/2023
- Generalized anxiety disorder - Onset: 11/30/2020 - continue Lexapro and Xanax
- Aortoiliac atherosclerosis - Onset: 12/31/2022 - ADM July , 2023 DC July 11, 2023 SVH DC Dx: Diverticulosis; UTI; AKI;

blood loss; hypokalemia; anemia; renal cyst; cholelithiasis; nephrolithiasis; rectal bleeding DC Meds: allopurinol 100 mg; alprazolam 0.25 mg q8 hrs as needed; amoxicillin-clav 875/125 bid; calcitriol 0.25 mcg qd; escitalopram 5 mg qd; pantoprazole 40 mg qd; pravastatin 40 mg qhs labs: glucose 122; K 4.0; egfr 34; ferritin 27.6; iron 258; hgb 10.4; Hct 32.7; plate normal; UA nitrite positiv; UCx >100,000 Kleb oxytoca; GI service called; Dx normo anemia with iron deficiency, rectal bleeding; on anticoagulation; diverticulosis; recommended CT Abdo and pelvis; if with continued "GI bleeding" then will proceed with colonoscopy. initiated on NSAIDS. CT Abdo and pelvis without contrast July 10, 2023 with: 1) BL renal cysts; 2 mm stone at mid pole L kidney 2) gallstone in the neck of the GB 1.7 cm 3) scattered diverticula without evidence of diverticulitis 4) trace bilateral pleural effusions 5) atherosclerosis of aorta CT Abdo and pelvis Nov 23, 2022 with: 1) atherosclerosis of aorta root 2) GB with calcified gallstone at the neck with GB distension 3) BL renal cysts 4) urinary bladder moderately distended without filling defect or wall thickening

- Uterine prolapse - Onset: 05/11/2023
- Cystocele - Onset: 06/28/2018 - Patient sees gyn every 2 months and has pessary changed.
- Proteinuria - Onset: 05/05/2021
- Advance directive discussed with patient - Onset: 01/15/2020
- Influenza vaccination declined - Onset: 11/30/2020 - SE to injection
- History of fall - Onset: 08/08/2022
- Chronic kidney disease stage 3 - Onset: 06/26/2018
- Low blood pressure - Onset: 12/31/2022
- Recurrent major depression in full remission - Onset: 05/05/2021
- Dementia - Onset: 05/11/2023
- Hypercoagulability state - Onset: 02/07/2019
- Bilateral hearing loss - Onset: 11/30/2020

Family History

Discussed Family History

Mother	- Arthritis
	- Depressive disorder
Father	- Depressive disorder

Social History

Discussed Social History
Health Risk Assessment
How would you rate your health?: Fair
Current Pain Level: (scale of 0 to 10; 10 being worse pain possible): 0
Do you have vision loss?: No
Do you have hearing loss?: Yes
Do you have urine leakage?: No
Do you exercise routinely?: No
If yes, how often do you exercise?: 0
What type of exercise?: 0
Have you fallen in the past three months?: No
If yes, please tell us the number of falls: 0
Do you need assistance walking?: Yes
Do you need any assistance bathing?: Full Assistance
Do you need any assistance dressing?: Some Assistance
Do you need any assistance eating?: Some Assistance
Do you need any assistance toileting?: Some Assistance
Do you need any assistance getting out of chairs?: Some Assistance
Do you need any assistance preparing meals?: Full Assistance
Have you had a dilated eye exam in the last 12 months?: No
Within the past 12 months, have you worried that your food would run out before you had money to buy more?: Never true
In the past 2 weeks, have you felt down, depressed, or hopeless?: No
In the past 2 weeks, have you been bothered by tiredness or fatigue?: No
In the past 2 weeks, have you felt stressed, anger, or social isolation?: No
In the past 2 weeks, have you been bothered by teeth or denture problems?: No
Do you have an Advanced Directive (i.e. living will, power of attorney, trust)?: No
If no, would you like to discuss this today?: No
If needed, who is able to assist with your care?: family
Do you need any assistance managing medications, grooming, using the phone, housekeeping, laundry, mode of transportation, or handling finances?: Some Assistance
Do you use a seat belt when in a vehicle?: Yes
Have you been recently hospitalized?: No
If Yes, Date of Last Admission and reason for hospitalization: 0
Within the last 12 months has a lack of reliable transportation kept you from Dr. appointments or getting things needed for daily living?: No
Within the past 12 months, has the food you purchased not lasted, and you didn't have money to buy more?: Never true
Quality Screening
Do you need any assistance managing your medications?: Full Assistance

Substance Use
Do you or have you ever smoked tobacco?: Never smoker
How many years have you smoked tobacco?: 20
At what age did you start smoking tobacco?: 17
How much tobacco do you smoke?: None (Notes: 1 cigarette / week)
When did you quit smoking?: 16+ years since last cigarette
Do you or have you ever used any other forms of tobacco or nicotine?: No
Do you or have you ever used e-cigarettes or vape?: Never used electronic cigarettes
Do you or have you ever used smokeless tobacco?: Never used smokeless tobacco
How much tobacco do you chew?: none
What was the date of your most recent tobacco screening?: 08/26/2025
Has tobacco cessation counseling been provided?: No
What is your level of alcohol consumption?: None
How many years have you consumed alcohol?: 0
Have you ever been counseled for unhealthy alcohol use?: No
Do you use any illicit or recreational drugs?: No
Which illicit or recreational drugs have you used?: none
What is your level of caffeine consumption?: Occasional
Advance Directive
Do you have an advance directive?: No
Do you need any assistance handling your finances?: Full Assistance
Do you need any assistance maintaining continence: No Assistance
Do you need any assistance with transferring?: Some Assistance
Exercise how many days per week?: 0
Education and Occupation
What is the highest grade or level of school you have completed or the highest degree you have received?: High school graduate
Are you currently employed?: No
What is your occupation?: retired
Relationships and Sexuality
What is your relationship status?: Widowed
Diet and Exercise
What type of diet are you following?: Regular
Do you have any dietary restrictions?: No
What is your exercise level?: None
Lifestyle
Over the last two weeks have you been feeling down, depressed, or hopeless?: Not at all
Over the last two weeks have you felt little interest or pleasure in doing things?: Not at all
Home and Environment
Have there been any changes to your family or social situation?: No
Are you passively exposed to smoke?: No
Public Health and Travel
Have you been to an area known to be high risk for COVID-19?: No
In the 14 days before symptom onset, have you had close contact with a laboratory-confirmed COVID-19 while that case was ill?: No
In the 14 days before symptom onset, have you had close contact with a person who is under investigation for COVID-19 while that person was ill?: No
Activities of Daily Living
Are you able to care for yourself independently?: No
Are you able to walk independently without assistance or assistive devices?: Yes: walks with assistive device(s) (Notes: Patient uses a walker as needed.)
Social History
Do you feel safe at home?: Yes
Other
If patient spent time in Wuhan City, China - Does the patient live in Wuhan City?: No
In the 14 days before symptom onset, did the patient spend time in Wuhan City, China?: No
Education: 12
Live alone or with others?: with others (Notes: Patient lives with son.)
Marital status: Widowed
Gender Identity and LGBTQ Identity
Gender identity: Identifies as Female
Assigned sex at birth: Female
Pronouns: she/her
First name used: GABY
Sexual orientation: Straight or heterosexual

Surgical & Procedure History

Discussed Surgical & Procedure History

no prior surgery

GYN History

Reviewed GYN History
Abnormal Pap: N.

Obstetric History

Reviewed Obstetric History

TOTAL	FULL	PRE	AB. I	AB. S	ECTOPICS	MULTIPLE	LIVING
2	2						2

Past Medical History

Discussed Past Medical History

HPI

97 year old lady here for ffup post ER visit.

UMC ER Dec 29, 2025
97 year old lady was found on the floor. with intermittent falls for the past 3 months. slight BL UE weakness---per ER likely due to deconditioning and difficulty following instructions on exam.

Dx: Fall; open nasal fracture; closed C6 spinous process fracture; elevated troponin; facial laceration;

meds upon dc: continued on eliquis 2.5 mg bid; pravastatin 40; pantoprazole 40 mg; calcitriol; allopurinol 100 mg 1/2 tab; donepezil 5 mg; escitalopram 5 mg; trazodone as needed qhs.

EKG Dec 26, 2025 with aflutter

labs Dec 26, 2025 with:
1) troponin 50, 44;
2) CK 282;
3) AST 46; ALT 12;
4) egfr 38;
5) normal cbc,

CT C Spine Dec 26, 2025 with:
1) acute minimally displaced C6 spinous process fracture
2) 1.4 cm hyperdense R parotid nodule---refer to ENT

CT Maxillofacial Dec 26, 2025 with:
1) comminuted fractures of nasal arch, nasal septum,
2) subcutaneous emphysema;
3) soft tissue swelling over the nares, bridge of nose;
4) L periorbital and frontal soft tissue swelling

CT Head Dec 26, 2025 without acute findings

son claims to stay home with patient.
declines placement in assisted/memory care due to cost.

---

most recently seen Dec 2, 2025
seen Aug 26, 2025 for MAWV
son declines losing P3HP as PCP, declining hospice care but willing to consider Palliative medicine only.
advance directives--no copy although son verbalizes wish for DNR. discussed today. POLST reviewed and signed.

even now, son is still in the process of acquiring guardianship ---patient has seen Neuro, and switched to Dr. G. Chopra from previous Neuro due to lack of access. she is required to have EEG, MRI, psychiatry clearance prior to Neuro agreeing to sign papers for guardianship.
in the meantime, son has verbalized patient to have DNR status. POLST done today reflects the same.

today, complains:
1) that memory seems to be worsening. unable to perform MMSE due to language barrier. Neurotrack screening--attempted today.
has hearing loss, but refuses to wear her hearing aids.
2) fell 3 weeks ago while home. son noted the day after that she had a bump on the left forehead above the brow. denies LOC, pain in back or hips or arm.

labs June 18, 2025 with:
1) pyuria and culture does not satisfy dx of UTI. asymptomatic

2) uric acid 3.4

comprehensive panel May 12, 20225 with:
1) egfr 45 from 51, 39, 41, 46, stable
2) LDL 79; HDL 62, TGL 55
3) uric acid 4.1

labs Oct 30, 2024 with:
1) UTI--augmentin rx sent

most recently seen June 25, 2024
comprehensive panel April 2, 2024 with:
1) egfr 51;
2) hypoproteinemia with hypoalbuminemia--increase protein PO intake. recheck 3 mos
3) LDL 94; HDL 71; TGL 72
4) normal cbc, LFT, TSH 1.82, alb/crea negative; PTH 63; uric acid 3.3; iron panel, ferritin
5) UTI--augmentin rx sent
labs Nov 21, 2023 with:
1) normalized cbc, with normalized iron level
labs Oct 24, 2023 with:
1) proteinuria, pyuria
2) egfr 39 from previous 41, 45, 42, 51, 52
3) low protein level with normal albumin
labs Sept 21, 2023 with:
1) hgb 11 from 11.4 in July 2023 and 13.5; and hct 33 from 34.5 and 39.9---with iron deficiency. keep GI please. trending down slowly--ferrous sulfate 325 mg qd. recheck 4-6 wk please
2) egfr 41 from 45, 42, 51, 52
3) iron 37 and ferritin 7
labs Aug 17, 2023 with:
1) egfr 46, from 42, 49, 48, stable
2) hypoproteinemia
3) LFT normalized with normal hepatitis panel
labs July 21, 2023 with:
1) urine culture with growth of yeast
2) hgb 11.4 and hct 34.5, normo normo with normal iron and ferritin
3) Mg 1.7
4) PTH 80
5) total iron 59, ferritin 27
labs at the hospital July 2023 with:
glucose 122; K 4.0; egfr 34; ferritin 27.6; iron 258; hgb 10.4; Hct 32.7; plate normal; UA nitrite positive; UCx >100,000 Kleb oxytoca;
Labs in hospital JUne 30, 2023 with:
sodium 146; Ca 8.3; albumin 2.8; TP 5.7; AST 68; ALT 20; phosphorus low; a1c 5.0; glucose 102; normal cbc; LDL 80; HDL 36; TGL 3
labs May 2, 2023 with:
1) proteinuria with cast
2) U Cx positive for UTI
labs April 28, 2023 with
1) TSH 1.82
2) normal cbc, cmp, uric
3) LDL 94; HDL 69; TGL 86
4) uric acid 4.2
CT Abdo and pelvis without contrast July 10, 2023 with:
1) BL renal cysts; 2 mm stone at mid pole L kidney
2) gallstone in the neck of the GB 1.7 cm
3) scattered diverticula without evidence of diverticulitis
4) trace bilateral pleural effusions
5) atherosclerosis of aorta
CTA June 21, 2023 without PE; trace BL pleural effusions; thoracic aortic atherosclerotic plaque and coronary artery calcification
LHc June 20, 2023 with non-obstuctive CAD. likely transient occlusion of RCA
Echo June 20, 2023 with EF 60-70%;
CXR June 20, 2023 with cardiomegaly and mild BL infiltrates. moderate aortic tortuosity and calcification.
CT Abdo and pelvis Nov 23, 2022 with:
1) atherosclerosis of aorta root
2) GB with calcified gallstone at the neck with GB distension
3) BL renal cysts
4) urinary bladder moderately distended without filling defect or wall thickening
flu vaccine advised q Oct ---family declines
Prevnar 20 overdue --family declines
mammo BL screening na as aged out
DEXA Dec 11, 2019 osteopenia
CRC screening--aged out

ophtha screening overdue
under care of Nephro for CKD 3a
under cardio care for afib, CAD, MI in July 2023
noted weight loss steadily. but as of today, compared to previous visit, weight is plateuing.
patient is actually functionally improved with PT---now able to move, and stand and more mobile. knee joints ROM have significantly improved---enough that the patient is verbalized how proud and happy she is that she can now flex and straighter her legs comfortably and able to sit cross-legged.
107.4 lbs With clothes(July 2022)
101.2 lbs With clothes(Sept 2023)
96.8 lbs With clothes (Oct 2023)
96.2 lbs with clothes (Dec 2023)
seen by GI Dr. Chen and was recommended colonography. when son made appt with radiology, per son, radiology recommended against this due to requirement of rectal tube insertion and pumping air and that patient needs to be in a hospital setting. because of potential complication, son and family opted to not do anything, and declined colonography.
at the same time, patient saw the gyne for pessary change q 3 months, and that according to gyne, per patient's son, the pessary is becoming more difficult to remove and they are recommended catheterization instead. patient's son is asking for clarification and maybe a second opinion.
unfortunatley, son wishes to continue with pessary and was referred to a Urologist. he is not satisfied with care by Dr. Grigorieve and requests a different uro-gyne.

no other complaints.

ROS

Additionally reports:
Constitutional: denies fever, chills/rigors, sweats, unexplained weight loss
Head: denies headache, dizziness
Eyes: denies diplopia, vision changes or loss, discharge, erythema/injection, eye pain, swelling, foreign body sensation
Ears: denies ear pain, discharge, hearing loss, swelling, erythema
Nose: denies epistaxis, discharge, pain
Neck: denies lymphadenopathy, thyroid enlargement, anosmia
Mouth: denies sorethroat, discharge, swallowing issues, choking
Chest: denies chest pain, palpitations, orthopnea,
paroxysmal nocturnal dyspnea(PND), dyspnea one exertion (DOE)
Respiratory: denies cough, sputum production, hemoptysis, cyanosis, shortness of breath
Abdomen: denies pain, bloating, nausea, vomiting, diarrhea, blood in stool, melena, hematemesis, coffee ground emesis, abdominal lumps or masses
Extremities: denies edema, calf pain, toe discoloration
Genitourinary: denies dysuria, frequency, urgency, hesitancy, hematuria.
denies urinary leakage
Neurologic: denies weakness, numbness, gait abnormality, falling in the recent 4-6 weeks
Psychiatric: denies depression, anxiety, suicidal ideations, homicidal ideations, hallucinations, delusions, access to firearms
Dermatology: denies skin lesions, wounds
Pain: denies neck pain, back pain, joint pain

Physical Exam

Constitutional: alert, awake, lucid, not in acute respiratory distress;
oriented to 3 spheres
HEENT: head movement supple and without limitations
no scalp wounds, cyanosis, facial or lid swelling
no conjunctival erythema
no visible thyroid enlargement
Respiratory/Chest: no visible distress.
CTA BL without rales nor wheezes
Cardio: S1 S2 IRIR
Abdomen: no visible bulging nor masses; no ostomy
Extremities: no edema, no discoloration
Skin: no visible lesions or rash
Neurological: CN 2-12 grossly intact; no visible sensorimotor deficits; cerebellar function intact
Gait: within normal limits

Assessment / Plan

1. Aortoiliac atherosclerosis -
asymptomatic
low cholesterol diet
statin

I70.0: Atherosclerosis of aorta

2. Chronic atrial fibrillation -
rate controlled.
continue anticoagulation and current regimen
I48.20: Chronic atrial fibrillation, unspecified

3. Anticoagulant therapy -
per cardio, afib with 1:4 aflutter, and need to continue anticoagulation
advised fall precautions.
Z79.01: Long term (current) use of anticoagulants

4. Tortuosity of branch of aortic arch -
asymptomatic
keep BP and cholesterol controlled
I77.1: Stricture of artery

5. Chronic kidney disease stage 3 -
no NSAIDS
slight deterioration
recheck 3 mos for stability

initiated on pantoprazole ---will watch renal function closely. not intended to take pantoprazole for more than 4-6 wks.
likely compounded by IV contrast at ER

acute ARI---increase fluid intake.
avoid caffeine
recheck 3 wks labs
if persistent, plan stop pantoprazole
N18.31: Chronic kidney disease, stage 3a
- CBC (INCLUDES DIFF/PLT)

6. Coronary atherosclerosis -
asymptomatic
continue current med regimen
I25.10: Atherosclerotic heart disease of native coronary artery without angina pectoris
- LIPID PANEL WITH REFLEX TO DIRECT LDL

7. Dementia -
stable
son as primary caretaker
F03.90: Unspecified dementia, unspecified severity, without behavioral disturbance, psychotic disturbance, mood disturbance, and anxiety

8. Generalized anxiety disorder -
taper alprazolam to dc while we titrate trazodone up
F41.1: Generalized anxiety disorder

9. Hypercoagulable state
continue eliquis.
asymptomatic

D68.69: Other thrombophilia
D68.59: Other primary thrombophilia

10. Old myocardial infarction -
asymptomatic
low cholesterol diet
I25.2: Old myocardial infarction

11. Hyperparathyroidism due to renal insufficiency -
asymptomatic
monitor
N25.81: Secondary hyperparathyroidism of renal origin
- COMPREHENSIVE METABOLIC PANEL
- PTH, INTACT (ICMA) AND IONIZED CALCIUM

12. Bilateral pleural effusion -
pending completion of RPT CXR
J90: Pleural effusion, not elsewhere classified

13. Hyperuricemia -
low purine diet
E79.0: Hyperuricemia without signs of inflammatory arthritis and tophaceous disease
- URIC ACID

14. Kidney stone -
adequate hydration
N20.0: Calculus of kidney

15. Recurrent major depression in full remission -
stable
continue escitalopram
F33.42: Major depressive disorder, recurrent, in full remission

16. Underweight -
Glucerna/ensure as snack
increase caloric intake
ff up nutritionist please
if with trend of weight loss again, rto immediately
R63.6: Underweight
- TSH

17. Spinal stenosis of lumbar region -
stable
avoid triggers
M48.061: Spinal stenosis, lumbar region without neurogenic claudication

18. Uterine prolapse -
pessary in place under gyne
N81.4: Uterovaginal prolapse, unspecified

19. Bilateral hearing loss
H91.93: Unspecified hearing loss, bilateral

20. Hormone replacement therapy -
for severe atrophic vaginitis rendering difficulty with removing the pessary.
patient's son aware of risks and potential complications of HRT--cva, Mi, dvt, pe
Z79.890: Hormone replacement therapy

21. Adult health examination
Z00.00: Encounter for general adult medical examination without abnormal findings
- EATING HEALTHY FOODS: CARE INSTRUCTIONS
- PREVENTING FALLS: CARE INSTRUCTIONS
- Patient Wellness Plan - Handout: Patient Wellness Plan

22. Advance directive discussed with patient -
advance directives discussed with son in visit with patient.
patient does not have advance directives, but patient has dementia. am reaching out to Cm to determine if there is a way of family and patient establishing and advance directives without need to go through POA and lawyers.

POLST reivewed, disccused and signed Oct 11, 2024
DNR
Z71.89: Other specified counseling
- ADVANCE CARE PLANNING: CARE INSTRUCTIONS

23. Active or passive immunization -
declines
Z23: Encounter for immunization

24. Screening for malignant neoplasm of breast
Z12.31: Encounter for screening mammogram for malignant neoplasm of breast
- LEARNING ABOUT BREAST CANCER SCREENING

25. Screening for osteoporosis
Z78.0: Asymptomatic menopausal state
- DUAL-ENERGY X-RAY ABSORPTIOMETRY (DXA) TEST: ABOUT THIS TEST
- DEXA SCAN SPINE HIPS

26. Ground-level fall
W18.30XA: Fall on same level, unspecified, initial encounter
R26.81: Unsteadiness on feet

27. Osteoarthritis of joint of bilateral hands - Onset: 11/30/2020
M19.041: Primary osteoarthritis, right hand
M19.042: Primary osteoarthritis, left hand

28. Osteopenia - Onset: 10/20/2023 -
Dexa Dec 23, 2019 with osteopenia
M85.80: Other specified disorders of bone density and structure, unspecified site

29. Transitioned from emergency department to self-care
Z78.9: Other specified health status

30. Fall, subsequent encounter
home health for 8-12 weeks only. subsequently transition to Outpatient PT.
her baseline was ambulation with cane.
wheelchair only for movement between car and doctor offices
W19.XXXD: Unspecified fall, subsequent encounter
R26.81: Unsteadiness on feet

- HOME HEALTH REFERRAL - Schedule Within: provider's discretion
  Note to Provider:
  referral processed , home health will reach out to patient to schedule.
  home health service for balance training--PT evaluation and treatment.
  from Jan 14, 2026 through March 14, 2026
  patient's son requests "Donna:
- LIGHTWEIGHT WHEELCHAIR - Use as directed. Qty: 1 Unit Refills: 0 Supplier: PREFERRED HOME CARE

31. Closed fracture of nasal bone with routine healing, subsequent encounter
CT C Spine Dec 26, 2025 with:1) acute minimally displaced C6 spinous process fracture
2) 1.4 cm hyperdense R parotid nodule---refer to ENT
CT Maxillofacial Dec 26, 2025 with: 1) comminuted fractures of nasal arch, nasal septum, 3) subcutaneous emphysema; 4) soft tissue swelling over the nares, bridge of nose; 5) L periorbital and frontal soft tissue swelling
CT Head Dec 26, 2025 without acute findings
S02.2XXD: Fracture of nasal bones, subsequent encounter for fracture with routine healing

- OTOLARYNGOLOGIST REFERRAL - Schedule Within: provider's discretion

32. Closed displaced fracture of sixth cervical vertebra with routine healing, unspecified fracture morphology, subsequent encounter
CT C Spine Dec 26, 2025 with:
1) acute minimally displaced C6 spinous process fracture
2) 1.4 cm hyperdense R parotid nodule---refer to ENT

CT Maxillofacial Dec 26, 2025 with: 1) comminuted fractures of nasal arch, nasal septum, 3) subcutaneous emphysema; 4) soft tissue swelling over the nares, bridge of nose; 5) L periorbital and frontal soft tissue swelling

CT Head Dec 26, 2025 without acute findings
S12.500D: Unspecified displaced fracture of sixth cervical vertebra, subsequent encounter for fracture with routine healing

33. Parotid nodule

CT C Spine Dec 26, 2025 with:
1) acute minimally displaced C6 spinous process fracture
2) 1.4 cm hyperdense R parotid nodule---refer to ENT

CT Maxillofacial Dec 26, 2025 with: 1) comminuted fractures of nasal arch, nasal septum, 3) subcutaneous emphysema; 4) soft tissue swelling over the nares, bridge of nose; 5) L periorbital and frontal soft tissue swelling

CT Head Dec 26, 2025 without acute findings
K11.8: Other diseases of salivary glands

- OTOLARYNGOLOGIST REFERRAL - Schedule Within: provider's discretion

34. Encounter for medication review
Z79.899: Other long term (current) drug therapy

35. Atrial fibrillation and flutter -
rate controlled.
continue eliquis and other meds.
no NSAIDS

Dr. Majidi plans to perform cardiac work-up July 2026 per patient's son's claim on her most recent visit.

I48.0: Paroxysmal atrial fibrillation

- Eliquis 2.5 mg tablet - TAKE ONE TABLET PO TWICE DAILY FOR A FIBRILLATION Qty: (180) tablet Refills: 2 DAW: Y Pharmacy: COSTCO PHARMACY # 685 Note to Pharmacy: 120 TABS
- CARDIOLOGIST REFERRAL - Schedule Within: provider's discretion Note to Provider: patient already saw Dr. Majidi post discharge from hospital. this referral is for continuity

36. Inadequate fluid intake

try to maintain 72 oz water per 24 hr

R63.8: Other symptoms and signs concerning food and fluid intake

Return to Office

- TERESA HONG, MD for M-FOLLOW-UP 30 at P3NV - 2831 Business Park on 04/07/2026 at 03:00 PM

Amendment Sign-Off

Encounter signed-off by TERESA HONG, MD, 01/14/2026.

Encounter performed and documented by TERESA HONG, MD
Encounter reviewed & signed by TERESA HONG, MD on 01/14/2026 at 02:01 PM
Amendment closed by TERESA HONG, MD on 01/14/2026 at 02:41 PM